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ARTICLE

Immediate Implants Placed in Infected and Noninfected Sites after Atraumatic Tooth Extraction and Placement with Ultrasonic Bone Surgery

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No conflict of interest.



Abstract

Background

Only a few reports deal with implants placed in infected postextraction sites.

Purpose

Survival rates of a cohort of immediate implants cases placed in acute and chronically infected sites were compared with a cohort of noninfected ones

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charge at infected sites.

Materials and Methods

Eighty-six patients received 168 immediate implants distributed into three groups: noninfected (85), acute (36), and chronically (47) infected sites. Atraumatic extraction and implant osteotomy were performed with an ultrasonic surgery device without flap elevation. All sites received the same medication and surgical protocol; infected sites were ultrasonicated during 30 seconds at 72 W. Kaplan-Meier survival rates were calculated at 1 year.

Results

The 1-year survival rates of the noninfected, chronically, and acute infected groups were 98.8, 100, and 94.4%, respectively. The differences were not statistically significant. No implant was lost after loading. All teeth and roots could be extracted in one piece. Drilling at extraction sockets was uncomplicated, without skidding.

Conclusions

Implant survival rates might be similar in infected and noninfected sites when infected sites receive standard medical and surgical treatment and are ultrasonicated. Atraumatic tooth/root extraction and implant placement can be reliably performed with piezoelectric surgery.

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